Agenda

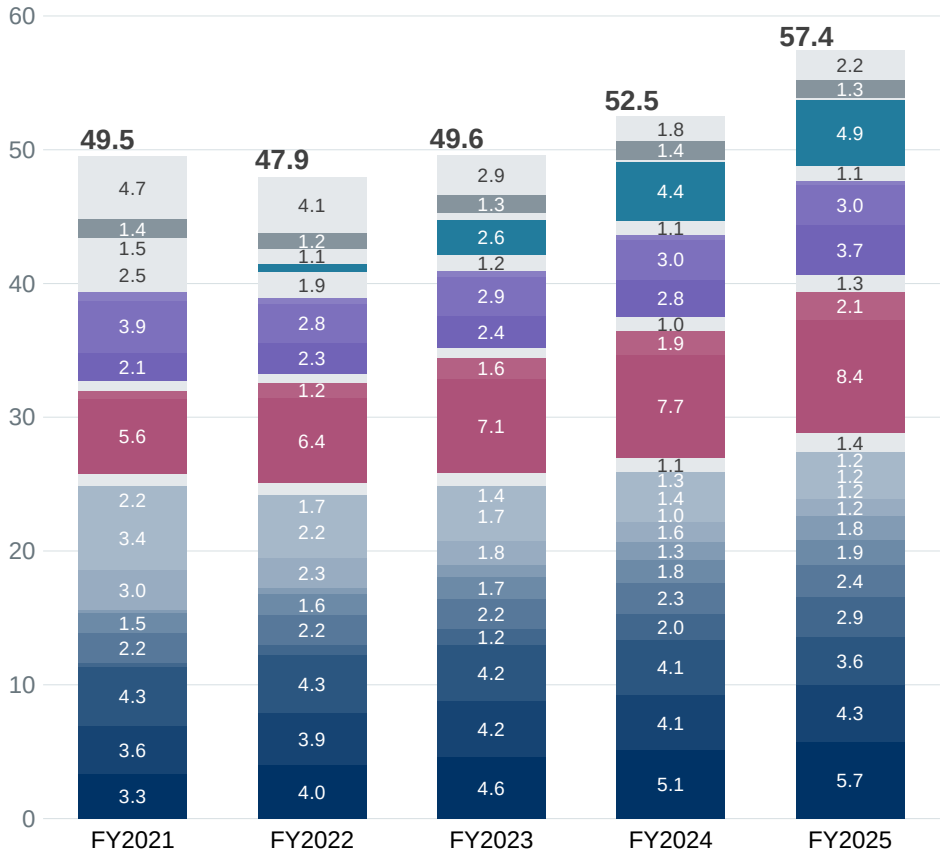


1. Eli Lilly (US)
2. Johnson & Johnson (US)
3. AbbVie (US)
4. **Roche (Switzerland)**
5. AstraZeneca (UK)
6. Merck & Co. (US)
7. Novartis (Switzerland)
8. Pfizer (US)
9. Sanofi (France)
10. Bristol Myers Squibb (US)
11. Novo Nordisk (Denmark)
12. GSK (UK)
13. Amgen (US)
14. Takeda (Japan)
15. Gilead (US)
16. Boehringer Ingelheim (Germany)
17. Bayer (Germany)
18. CSL (Australia)
19. Viatris (US)
20. Teva (Israel)

Commercial portfolio: Oncology/Hematology contributed 50% of FY2025 Pharma sales, while Ocrevus remained the largest product at $8.4bn

Roche Pharma global product sales | FY2021–2025

USD bn | total CAGR +3.8%



Core products | FY2025 sales > USD1bn

Product / family	CAGR	MoA
Activase/TNKase	-2%	tPA fibrinolytics
Vabysmo	n.m.	VEGF-A × Ang-2 bsAb
MabThera/Rituxan (Immuno)	-14%	Anti-CD20 mAb
Actemra/RoActemra	-7%	Anti-IL-6R mAb
Xolair	+15%	Anti-IgE mAb
Evrysdi	+34%	SMN2 splicing modifier
Ocrevus	+11%	Anti-CD20 mAb
MabThera/Rituxan (Onc/Heme)	-15%	Anti-CD20 mAb
Avastin	-23%	Anti-VEGF-A mAb
Gazyva/Gazyvaro	+12%	Anti-CD20 mAb
Herceptin	-20%	Anti-HER2 mAb
Polivy	+60%	CD79b-directed ADC
Alecensa	+6%	ALK inhibitor
Kadcyla	+3%	HER2-directed ADC
Phesgo	+67%	Anti-HER2 mAbs combination, SC
Perjeta	-5%	Anti-HER2 mAb
Tecentriq	+4%	Anti-PD-L1 mAb
Hemlibra	+15%	FIXa × FX bsAb
Others (all TAs)		Remaining products

Oncology/Hematology

FY2025	FY2026H1
$28.8bn | 50.2%	$15.1bn | 50.6%
CAGR +2.8%	Onc +1% / Heme +14%

Neurology

FY2025	FY2026H1
$11.8bn | 20.6%	$6.3bn | 21.0%
CAGR +14.5%	CER +9%

Immunology

FY2025	FY2026H1
$8.1bn | 14.1%	$4.2bn | 13.9%
CAGR -3.1%	CER +8%

Ophthalmology

FY2025	FY2026H1
$5.1bn | 8.8%	$2.7bn | 8.9%
CAGR +35.9%*	CER +6%

Other

FY2025	FY2026H1
$3.6bn | 6.2%	$1.6bn | 5.5%
CAGR -12.6%	CER not disclosed

*2021 Ophthalmology not restated for Luxturna. MabThera is allocated across two TAs: 17 families / 18 named chart segments. H1 is not annualised.

Source: AR2022 pp15–16; FR2023 pp16–17; FR2025 pp15–17; FY25 slides140; H1 results39–46/133. USD: average CHF/USD; CAGR: reported USD, not CER.

Late-stage portfolio: Roche's 13 Phase III and 3 registration NME buckets represent 15 unique molecules after removing the giredestrant overlap

15 Unique company-coded late-stage NMEs

12 Phase III at highest stage

3 Registration at highest stage

TA	Modality	Asset / stage	MoA	Origin / partner	Indications in highest stage
Oncology / Hematology	Small molecule	giredestrant Registration	Oral ER degrader / SERD	Roche / Genentech	ER+ adjuvant BC; post-CDK4/6 ER+/HER2− BC + everolimus
Neurology	Small molecule	fenebrutinib Registration	Reversible BTK inhibitor	Roche / Genentech	Relapsing MS; primary progressive MS
Ophthal.	Antibody	vamikibart Registration	Anti-IL-6 antibody, intravitreal	Roche / Genentech	Uveitic macular edema
Oncology / Hematology	TCE / bsAb	cevostamab	FcRH5 × CD3	Roche / Genentech	Relapsed/refractory multiple myeloma
Oncology / Hematology	Small molecule	divarasib	KRAS G12C inhibitor	Roche / Genentech	2L NSCLC; 1L NSCLC + pembrolizumab; resected stage II–III NSCLC
Oncology / Hematology	bsAb	zemocimig (NXT007)	FIXa × FX coagulation-factor mimetic	Chugai	Hemophilia A
Oncology / Hematology	Degrader	bexobrutideg	BTK degrader	Nurix	Relapsed/refractory CLL
Immunology	ASO	sefaxersen	Complement factor B mRNA	Ionis	IgA nephropathy
Immunology	mAb	afimkibart	Anti-TL1A	Telavant	Ulcerative colitis; Crohn's disease
CVRM	siRNA	zilebesiran	Angiotensinogen (AGT) mRNA	Alnylam	Hypertension
CVRM	Peptide	enicepatide (CT-388)	Dual GLP-1 / GIP receptor agonist	Carmot	Obesity with/without type 2 diabetes
CVRM	Peptide	petrelintide	Long-acting amylin analogue	Zealand Pharma	Obesity with/without type 2 diabetes
CVRM	Protein analogue	pegozafermin	FGF21 analogue	89bio	MASH with fibrosis F2/F3; MASH with cirrhosis F4
Neurology	Shuttle Ab	trontinemab	Amyloid-β antibody with TfR1 shuttle	Roche	Early Alzheimer's disease; preclinical Alzheimer's disease
Neurology	mAb	prasinezumab	Anti-α-synuclein	Prothena	Parkinson's disease

Review flag: astegolimab (anti-ST2; Phase III COPD) is shaded AI in the company table. Excel retains this classification exception; approved/LCM status is not inferred.

Source: Q3 appendix p51, status 22 Oct 2026; Pharma Day programme pages. NME/AI coding follows company shading; mechanism annotations supplementary.

R&D priorities: Roche defines distinct strategies across five therapeutic areas, supported by a common “Bar” for portfolio investment

Oncology / Hematology	Neurology	Immunology	Ophthalmology	CVRM
Achieve leadership through transformative anchors	**Extend the #1 position beyond 2030**	**Maximize value through multi-indication expansion**	**Become #1 in retina**	**Secure a competitive entry before 2030**
Establish and expand key patient segments	Broaden the portfolio and lead in preventive neurology	Use pan-immunology pathways and cross-TA synergies	Build a diversified, differentiated pipeline	Differentiate through Roche capabilities
Secure position with the next wave of innovation	Integrate Pharma + Diagnostics across the patient journey	Pursue curative therapies for sustained remission	Expand beyond retinal vascular disease into dry eye and glaucoma	Become a top-three player in obesity
Selected programme examples	**Selected programme examples**	**Selected programme examples**	**Selected programme examples**	**Selected programme examples**
giredestrant (oral SERD) divarasib (KRAS G12C inhibitor) bexobrutideg (BTK degrader)	fenebrutinib (BTK inhibitor) trontinemab (amyloid-β Brainshuttle Ab) prasinezumab (anti-α-synuclein)	afimkibart (anti-TL1A) Gazyva (anti-CD20) sefaxersen (factor B ASO)	Vabysmo (VEGF-A×Ang-2) vamikibart (anti-IL-6) VEGF×IL-6 DutaFab	enicepatide (GLP-1/GIP agonist) petrelintide (amylin analogue) pegozafermin (FGF21 analogue)

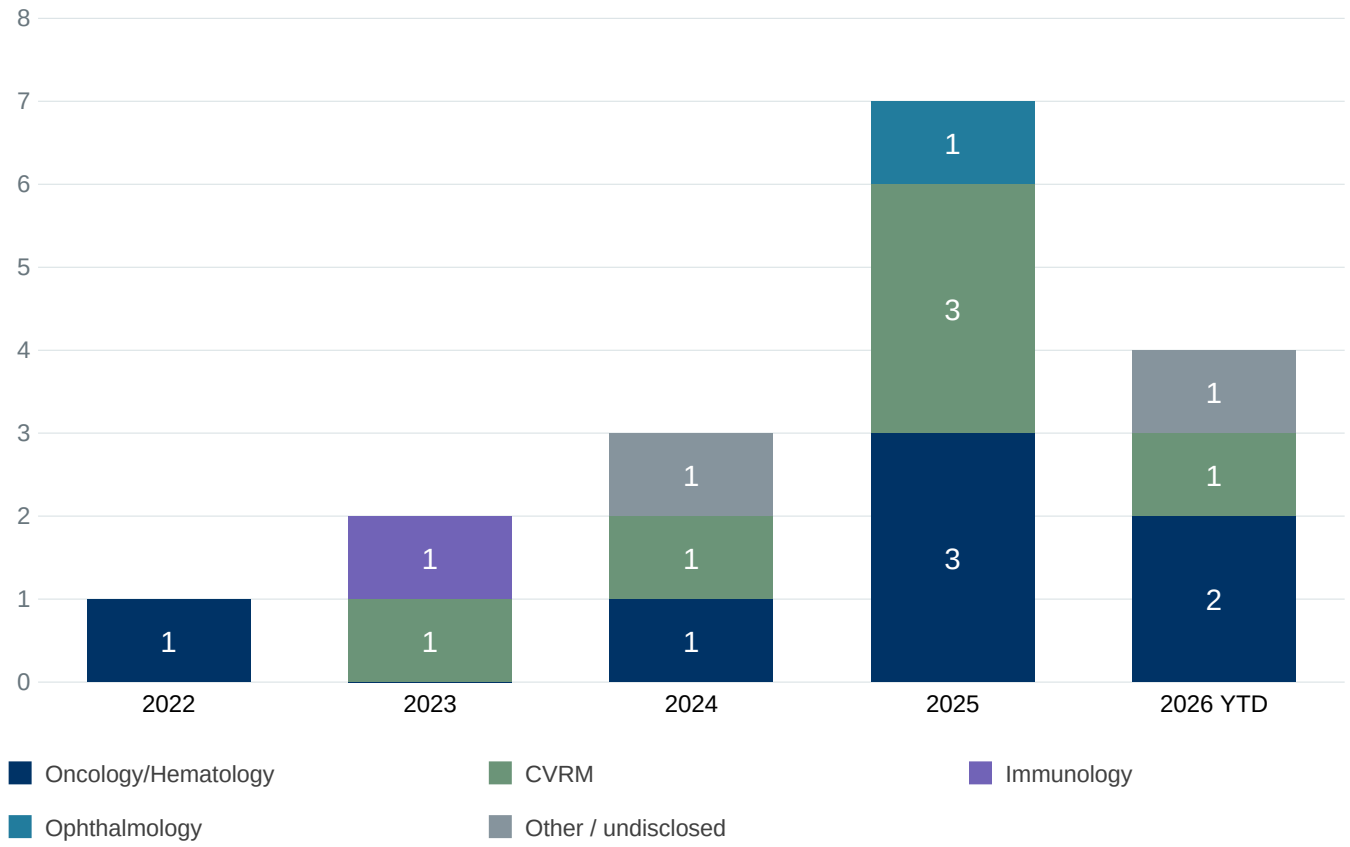
R&D Excellence: 20 transformative medicines by 2030; “Bar” criteria cover unmet need, foundational target, drug quality, differentiation and value.

Source: Pharma Day 2026 pp15,47–48 and TA programme sections. Goals are company-disclosed; selected programme examples illustrate those goals.

BD partnering: Selected disclosed transactions add CVRM mechanisms and oncology platforms through acquisitions and licensing collaborations

17 selected inbound transactions | 2022–2026 YTD

One transaction per row; multi-asset acquisitions counted once. 2026 is year-to-date.



What the disclosed deals add to the portfolio

CVRM

Carmot → enicepatide / CT-996
Zealand → petrelintide
89bio → pegozafermin

Oncology/Hematology

Regor → CDK inhibitors
Poseida → allogeneic CAR-T
MediLink → B7-H3 ADC

Immunology

Telavant → afimkibart (anti-TL1A)
US/Japan commercialization rights

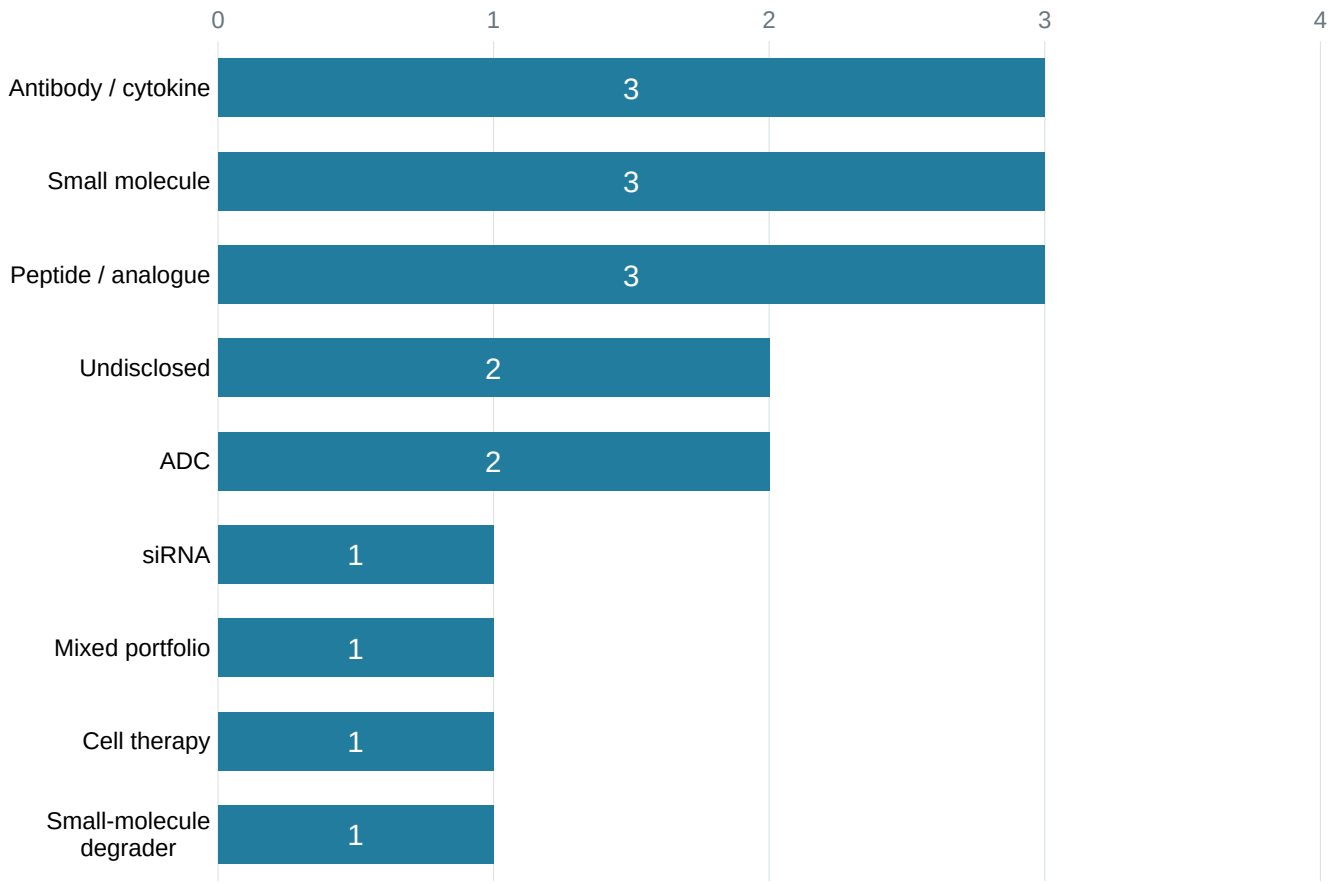
Ophthalmology

Pentavision → Phase I trispecific antibody
Targets not named in acquisition note

Source: AR2022 p68; FR2023 pp70/81; FR2025 pp10/72–76/88; Pharma Day p56/198; H1 results p6. Selected documented cohort, not all Roche BD.

BD partnering: The selected deal cohort spans small molecules, peptides, ADCs, cell therapy and RNA modalities, with targets disclosed at varying levels

Selected transactions by primary modality



Targets / mechanisms represented in the cohort

Oncology/Hematology

BTK degradation • B7-H3 ADC
CDK4/2 • BCMA CAR-T • PD-1/IL-2

CVRM

GLP-1/GIP • amylin • FGF21
AGT siRNA • CRFR2/UCN2 • ETA/AT1

Immunology

TL1A blockade
Telavant / afimkibart

Ophthalmology

Trispecific antibody platform
Individual targets not disclosed

Other / undisclosed

AntlerA / Simcere: reviewed materials do not identify the target/programme

Source: Same selected 17-transaction cohort as prior page. One primary modality per transaction; mixed and undisclosed portfolios remain explicit.

BD partnering: 2026 disclosures add a clinical BTK degrader, an ADC platform and a new obesity mechanism; some programme details remain undisclosed

Selected 2026 inbound transactions

Partner	Transaction / TA	Asset / MoA / modality	Development / transaction status	Evidence boundary
Nurix	Collaboration Oncology/Hematology	bexobrutideg Small-molecule BTK degrader	Clinical at deal; Phase III in 2L CLL by Pharma Day	First-line combinations and MS/CSU development discussed; separate from fenebrutinib.
DualityBio	Platform collaboration Oncology/Hematology	ADC discovery platform Individual targets not specified	Company-disclosed 2026 deal; discovery access	No named ADC or clinical stage assigned where reviewed material does not identify one.
Hanmi	Collaboration CVRM	HM17321 / RG7985 CRFR2-selective UCN2 analogue	Pending deal closure at Pharma Day; Phase I listed in pipeline	Body-composition rationale supported by preclinical data; no clinical efficacy inference.
Simcere	Partnering Programme undisclosed	Asset / modality / target not specified in reviewed timeline	Partner and 2026 year shown in company timeline	Terms and programme identity not inferred from the company logo.

2025 transactions feeding the current development portfolio

89bio

pegozafermin (FGF21 analogue)
Acquired Phase III MASH asset

Zealand Pharma

petrelintide (amylin analogue)
Obesity monotherapy and combinations

Poseida

Allogeneic CAR-T platform
Clinical/preclinical assets and capabilities

Source: H1 results p6; Pharma Day pp56/138/198/203 and transcript P0393–396/P0506–508. Dates remain years when exact announcement days are not provided.

Earnings calls summary: Pharma Day and H1 discussions highlight future combinations, preventive neurology and portfolio choices in obesity

Pharma Day | 28 September 2026

Build giredestrant combinations and MRD-guided adjuvant trials

Develop giredestrant (oral SERD) with CDK inhibitors, including a Pfizer CDK4 inhibitor; launch AFT-70 NAVIGATE with Alliance/Natera to tailor treatment using ctDNA/MRD.

Test a BTK degrader beyond established BTK inhibition

Begin Phase III Daybreak 306 for bexobrutideg (BTK degrader) in 2L CLL; build combinations to move toward first line and explore MS/CSU development.

Move Alzheimer's development into prevention

Develop trontinemab (amyloid-β / TfR1 Brainshuttle antibody) in early AD via TRONTIER-1/-2 and in pre-symptomatic disease via PrevenTRON; support recruitment with pTau217 screening.

Broaden incretin programmes and test combinations

Plan enicepatide (GLP-1/GIP agonist) head-to-head testing versus tirzepatide in glycaemic control, plus cardiovascular/heart-failure outcomes studies; combine with petrelintide (amylin analogue).

Prioritize body composition in obesity

Add Hanmi's UCN2 analogue (CRFR2 agonist; pending deal closure) to target fat loss with lean-mass preservation. Management explicitly linked this portfolio choice to stopping emugrobart.

Combine vascular and inflammatory targets in retina

Advance VEGF×IL-6 DutaFab toward Phase III in DME; broaden ocular inflammation treatment through vamikibart (anti-IL-6) in UME and Enspryng (anti-IL-6R) in TED.

H1 2026 earnings call | July 2026

Expand the giredestrant patient population

Generate giredestrant (oral SERD) data for lower-risk early breast cancer, patients switching from CDK4/6 inhibitors and new combinations; management framed these as gaps beyond the initial trial population.

Move divarasib into first-line combination treatment

Following positive head-to-head Phase III results in 2L+ NSCLC, advance divarasib (KRAS G12C inhibitor) + pembrolizumab (anti-PD-1) in first-line NSCLC via KRASCENDO-2.

Prepare MS filings and preventive AD development

Prioritize US filing for fenebrutinib (BTK inhibitor), followed by other markets; advance trontinemab (amyloid-β Brainshuttle antibody) and the pre-symptomatic AD programme.

Extend B-cell depletion into renal autoimmunity

Pursue Gazyva (anti-CD20) in SLE, membranous nephropathy and childhood nephrotic syndrome; evaluate bexobrutideg (BTK degrader) for differentiated activity in immune diseases.

Build multi-mechanism obesity treatment

Advance petrelintide (amylin analogue) toward Phase III, broaden enicepatide (GLP-1/GIP agonist) development and initiate their combination study; await CT-996 (oral GLP-1 agonist) data.

Expand beyond anti-VEGF retinal treatment

Seek new ocular indications for Enspryng (anti-IL-6R; TED) and vamikibart (anti-IL-6; UME), alongside continued Vabysmo (VEGF-A×Ang-2) development.

Source: User-provided Roche Pharma Day transcript (Sep2026) and H1 earnings-call transcript (Jul2026). Full original paragraphs and references are in the evidence workbook.